Ambulatory Care · Chronic Disease Management · Clinical Informatics

Andrew Tran, PharmD, BCACP, CDCES

Clinical Pharmacist — Ambulatory Care & Value-Based Care

I bring value-based care to everything I do. My passion has always been patient care and teaching — the relationships I build along the way are what drive me. I enjoy building systems and partnerships that get results, and I'm always looking for ways to simplify complex ideas so they're easier for patients, colleagues, and healthcare teams to understand.

View CV Portfolio

A little about

Background

I've been a clinical pharmacist for over seven years across a variety of settings — federally qualified health centers (FQHCs), anticoagulation clinics, community-oriented primary care clinics (COPCs), specialty mail-order operations, and telehealth. I completed my PGY-1 Community Pharmacy residency with a specialization in Ambulatory Care. My experience spans chronic disease management under collaborative practice agreements

(CPAs), protocol development, clinical tools & workflow creation, multi-state pharmacy law compliance, specialty pharmacy accreditation standards, and patient education.

PGY-1 Residency

Ambulatory Care

7 years

Chronic disease management

4 states

TX · OR · MI · LA licensure

BCACP · CDCES

Board certified

Impact across roles

Baylor Scott & White

$1.46M

Cost avoidance from a pharmacist-led transitions-of-care program

Baylor Scott & White

26%

Improvement in anticoagulation time-in-therapeutic-range

Parkland Health

2.3%

A1c reduction across a 600–700 patient CPA panel

Stellus Rx

$278 PMPM

Decrease in total cost of care within the Engage model

Experience timeline

Hover or tap a role to see highlights

- Engage Clinical Pharmacist· Stellus Rx · Remote · Aug 2026 – Present▾

  - Provide comprehensive medication management (CMM) through telephonic and virtual/video patient encounters within Stellus Rx's Engage model — a pharmacist-led program shown across three independent studies (750+ patients per cohort) to reduce total cost of care by $278 PMPM and cut hospital readmissions by 41% — identifying and resolving drug therapy problems while reconciling medications to support safe transitions of care following hospital discharge.
  - Design individualized therapeutic regimens and chronic disease state management action plans with measurable clinical goals — contributing to a 25% decrease in the A1c-uncontrolled cohort and a 21% reduction in ER visits — while conducting adherence assessments and reinforcing guideline-directed therapy per each patient's Plan of Care.
  - Collaborate with physicians, health systems, and the care coordination team to develop and refine medication management protocols, documenting comprehensive medication reviews in the Engage platform and EnterpriseRx in compliance with regulatory standards.

- Ambulatory Clinical Pharmacist I–III· Baylor Scott & White Health · Temple, TX · Jul 2023 – Mar 2026▾

    - Managed a chronic disease management panel of ~1,100 patients across a 30-mile service radius under a collaborative practice agreement, averaging 40 patient medication interventions daily — point-of-care visits, peri-operative planning, and discharge follow-up.

    - Clinical informatics lead for the anticoagulation service line — authored a 75-page Protocols and Procedures manual, updated the collaborative practice agreement governing drug therapy management, and standardized the clinical workflow.

    - Drove a 26% improvement in time in therapeutic range and a 16% increase in guideline-directed statin prescribing; supported a transitions-of-care program that cut 30-day readmissions by 13 points and generated ~$1.46M in cost avoidance.

- Specialty Clinical Pharmacist· Ascension Health · Austin, TX · Feb 2022 – Feb 2023▾

    - Piloted and launched Ascension's new specialty pharmacy service line in Austin; the protocols and workflows were later adopted nationally across 38 states.

    - Served as Pharmacist-in-Charge across 3 states, maintaining active multi-state licensure (TX, MI, OR, LA).

    - Delivered virtual pharmaceutical care for complex, chronic, and rare

> conditions — oncology, rheumatology, dermatology, GI, hepatitis C, HIV — performing 30–40 patient calls daily.

> - Ambulatory Care Clinical Pharmacy Specialist· Parkland Health & Hospital System · Dallas, TX · Feb 2021 – Feb 2022▾

> - Managed a primary care panel of ~600–700 patients under a collaborative practice agreement spanning 8 providers, independently initiating and titrating therapy for diabetes, hypertension, dyslipidemia, anticoagulation, heart failure, and asthma/COPD.
> - Contributed to A1c reductions of up to 2.3%, LDL reductions of up to 9.1 mg/dL, and guideline-directed diabetes therapy rates above 74% (ACEI/ARB) and 83% (statin).
> - Facilitated Healthy Living with Diabetes group education classes and served as a medication access resource for an underserved patient population.

> - Clinical Pharmacist· Baylor Scott & White Health · Temple, TX · Jul 2019 – Jan 2021▾

> - Delivered clinical pharmaceutical care to an outpatient clinic population, consulting with nursing and medical staff on medication orders and therapy in accordance with hospital policy.
> - Served as a clinical resource for pharmacy students and residents in a fast-paced outpatient clinic environment.

- PGY-1 Community Pharmacy Resident· Penobscot Community Health Care (PCHC) · Bangor, ME · Jun 2018 – Jun 2019▾

  - Delivered comprehensive medication management for a chronic disease panel of 15–30 patients per day across ambulatory rotations at three FQHC sites.

  - Completed 460+ hours of community pharmacy staffing alongside rotations in Transitions of Care, Administration, Population Health Management, and Patient-Centered Dispensing.

  - Led a research project on DOAC utilization and implemented a pharmacist-led DOAC telephone clinic; served on the Controlled Substance Stewardship and High Utilizer Group committees.

  See full residency training →

- Staff Pharmacist· Walmart Pharmacy · Gatesville, TX · Aug 2017 – May 2018▾

  - Led a team of 6 pharmacy technicians in accurately processing and dispensing a weekly volume of 1,500–1,800 prescriptions, counseling patients on medication use, safety, and healthcare needs.

  - Administered immunizations and company-sponsored health programs while ensuring full compliance with HIPAA, PHI, and regulatory standards.

- Doctor of Pharmacy· Texas A&M University, Irma Lerma Rangel College of Pharmacy · Kingsville, TX · 2013 – 2017▾

  Coursework

  Forum & Professional Development I–III; IPPE I–III & Institutional Pharmacy Practice Experience; Principles of Drug Action I & II; Human Physiology; Biochemistry; Pharmaceutical Calculations; Health Care Systems; Clinical Communications; Introduction to Patient Care; Research Methods & Biostatistics; Pharmaceutics I & II; Pharmacy Law & Ethics; Public Health & Pharmacoepidemiology; Self-Care & Non-Prescription Medications; Electrolytes, Acid-Base, Anemias & Kidney Disease; Cardiovascular Diseases; Interprofessional Team Recitation/Rounds I–IV; Nutrition, Vitamins, Complementary & Alternative Medicine; Basic Pharmacokinetics; Sterile Products & IV Admixtures; Anticoagulation Therapy & Management; Endocrinology & Metabolism; Neurology & Pain Management; Microbiology & Immunology; Pharmacy Management; Drug Literature Evaluation & Patient Drug Education; Health Policy & Management; Spanish for Pharmacists; Psychiatry & Addiction; Critical Care, GI, Pulmonary & Rheumatology; Toxicology & Poison Management; Patient Assessment; Pharmacoeconomics; Infectious Diseases; Oncology, Transplant & Genomic Medicine; Pharmaceutical Care Lab; Social & Behavioral Aspects of Patient Care; Clinical Pharmacokinetics; Grand Rounds II Capstone.

  APPE Rotations — 4th Year

  - Internal Medicine — Seton Medical Center Williamson, Round Rock, TX · Jul – Aug 2016 · Preceptor: Ladan Panahi, PharmD, BCPS

  - Community Pharmacy — Scott & White Health Plan Pharmacy, Round Rock, TX · Aug – Sep 2016 · Preceptor: Tejas Patel, PharmD

- Ambulatory Care — Scott & White Healthcare Clinic, Georgetown, TX · Sep – Oct 2016 · Preceptor: Heather Hay, PharmD, BCPS, BCACP, CDE
- Hospital Systems — Scott & White Medical Center, Round Rock, TX · Nov – Dec 2016 · Preceptor: Melanie Moss, PharmD, BCPS
- Regulatory Affairs — Texas State Board of Pharmacy, Austin, TX · Feb – Mar 2017 · Preceptor: Synthia Hill, PharmD
- Professional Pharmacy Association — Texas Pharmacy Association, Austin, TX · Mar – Apr 2017 · Preceptor: Kim Roberson, RPh

Final GPA: 3.51 · Honors: Cum Laude

The foundations

What I Do

01

Chronic Disease Management

I'm board certified in ambulatory care (BCACP) with residency training in chronic disease management — diabetes, hypertension, heart disease, CHF, anticoagulation, asthma/COPD — plus transitions of care, specialty pharmacy, and controlled substance stewardship. I have over 7 years of direct patient care experience managing chronic disease states under collaborative practice agreements, via outpatient and telehealth visits.

02

Protocol Design

I have extensive experience with collaborative practice agreements and developing clinical workflows and protocols. I've contributed to business plans, practice agreements, and protocols for multiple service lines.

03

Medication Education

I obtained my teaching certificate from the University of New England College of Pharmacy and am also a Certified Diabetes Care and Education Specialist (CDCES). I have also precepted pharmacy students from UNE, Husson, University of Texas, Texas Tech, and Texas A&M Colleges of Pharmacy.

04

Clinical Tools & Informatics

I leverage technology to deliver high-quality clinical care — including a perioperative anticoagulation bridge plan maker and a diabetes treatment algorithm tool.

Selected work

Projects

Click or tap a thumbnail to see it larger

Clinical Tool

Diabetes Treatment Algorithm Tool

Built and maintained by Andrew Tran, PharmD (2026)

A clinical decision-support tool built in Excel, updated to the most recent American Diabetes Association guidelines — assists in selecting diabetes medications and calculating insulin dosing.

Diabetes management Insulin dosing ADA guidelines

Download .xlsx ⬇

Diabetes Treatment Algorithm — Patient Information

Red = enter data here | Blue = calculated automatically | Green = updated for ADA S

Disclaimer: use of this tool does not replace clinical judgment.

Input Patient Parameters	
Patient Age (years)	
Gender (male, female)	
Total Body Weight (kg)	
Height (in)	
Ideal Body Weight (kg)	
Adjusted Body Weight (kg)	
Patient's BMI (%)	
Weight used in CrCl calculation	
Serum Creatinine (mg/L)	
CrCl via Cockcroft-Gault Equation (mL/min)	
EGFR (mL/min)	

Clinical Tool

Anticoagulation Bridge Plan Maker

Originally built 2011 · redesigned and maintained by Andrew Tran, PharmD (2026)

Excel tool that automates perioperative anticoagulation bridge therapy planning — generates a day-by-day Lovenox/warfarin dosing schedule around a surgery date and includes a built-in creatinine clearance calculator.

CrCl calculator 14-day bridge planning

Download .xlsx ⬇

CrCl Calculator	
Height(in.):	
Weight (kg):	
Age:	
SCr (mg/dL):	
Male/Female:	

IBW:		*kg*	*Height to Male/Female must be filled*
TBW/IBW			****If SCr is less than 1.0, a value of 1.0 will be used in the CrCl formula****
Adj. BW:		*kg*	
CrCl (IBW):		*mL/min*	
CrCl (ABW):		*mL/min*	

MRN	Last name	First Name	Stop Warfarin On:	Surgery Date	Lab Date:	Lab Loc:

Protocol / Clinical Systems Design

BSWH Anticoagulation Clinical Protocol and Procedures

Baylor Scott & White Health — Temple, TX · Updated January 2026

Sole author and developer of the clinic's protocol and procedures manual — revising, combining, and updating procedural documents spanning 1997–2023 into one comprehensive document to streamline and standardize responsibilities across the anticoagulation clinic.

Sole author 75-page protocol 1,100 patients

Protocol development Clinical workflow design

BaylorScott&White HEALTH

Baylor Scott & White Anticoagulation Clinic Protocol and Procedures

Updated: January 2026

Developed by: Andrew Tran, Clinical Pharmacist III

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Compliance / Regulatory Systems

Multi-State Pharmacy Law Compliance Database

Built and maintained by Andrew Tran, PharmD · Updated August 2026

Personal reference database covering pharmacist-in-charge, pharmacy/pharmacist licensure, controlled-substance, and prescription-label requirements across all 50 states + DC — built to support multi-state licensing and compliance work.

PIC · 3 states Pod Lead

Clinical workflow design Protocol development

Download .xlsx (read-only) ⬇

National Rollout

Ascension Specialty Rx — Pharmacy Team Procedures

Ascension Health - Austin, TX

Contributor on the pioneering Ascension Specialty Mail Order team — developed the combined pharmacist and pharmacy-technician procedures manual governing Therigy workflows, patient onboarding, benefits/adjudication, and day-to-day operations for the specialty pharmacy team.

38 states 35-page combined manual

Clinical workflow design Protocol development

Ascension Rx - Austin Specialty Pharmacist Procedure

Specialty Pharmacy Team Procedures – Version 1.0

Updated: 10/06/2022

Contributors:

George Butchko, BSPharm; Daniel Elliot, PharmD; Ndinda Domingos PharmD; Margaret Sant PharmD; Cylene Ann Talabucon, PharmD, Andrew Tran, PharmD

Elizabeth Lopez, CPhT; Jesse Morquecho, CPhT Hector Rodriguez, CPhT; Melvin Sanchez, CPhT; Andrea Soliz-Garrett, CPhT

Table of Contents

Business Proposal

Business Plan for Clinical Pharmacy Service

Waco Family Medicine, in conjunction with Texas A&M College of Pharmacy · October 2021

Proposal to implement an ambulatory care pharmacy service line at an FQHC in Waco, Texas — an ambulatory care pharmacist position co-founded by Texas A&M College of Pharmacy and Waco Family Medicine to see patients and precept pharmacy students.

Chronic disease management Medication adherence

Business planning Pharmacy advocacy Pharmacy education

WACO FAMILY MEDICINE

Business Plan for
Clinical Pharmacy Service

Plan prepared by
Andrew Tran, PharmD, BCPS, BCACP

October 2021

PGY-1 Residency Research Project

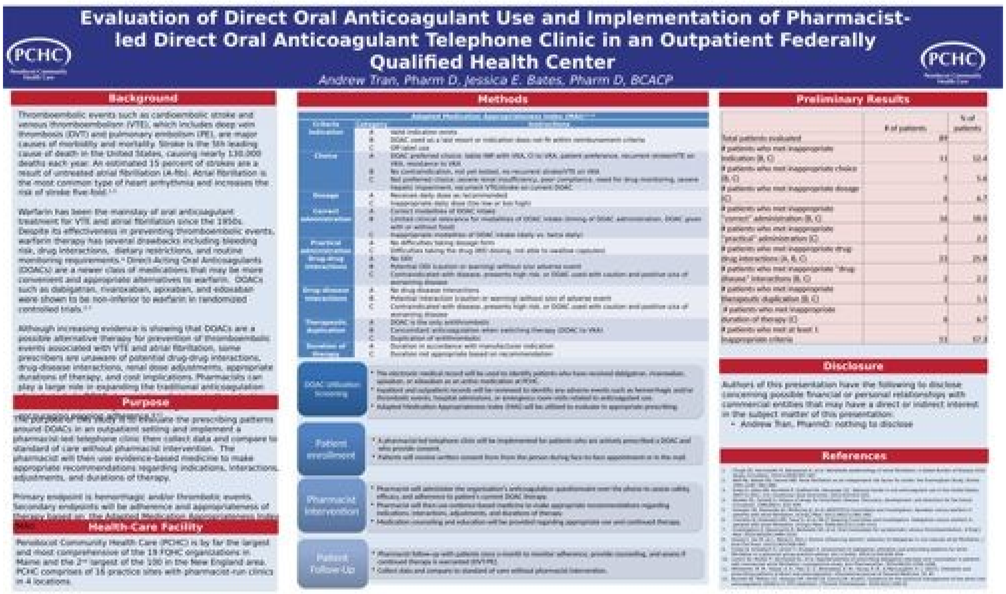
Evaluation of DOAC Use & Pharmacist-Led DOAC Telephone Clinic in an FQHC

Penobscot Community Health Care, Bangor, ME · Presented at ASHP Midyear Clinical Meeting, Nov 2018 · co-authored with Jessica E. Bates, PharmD, BCACP

Residency research project evaluating prescribing appropriateness of direct oral anticoagulants against the Adapted Medication Appropriateness Index, and implementing a pharmacist-led DOAC telephone clinic in an outpatient FQHC.

89 patients evaluated 57.3% had ≥1 inappropriate criterion

Residency research Anticoagulation



Published Research

Loperamide Misuse and Abuse

Journal of the American Pharmacists Association (JAPhA), 2017 · co-authored with H. Miller, L. Panahi, D. Tapia, J.D. Bowman

Peer-reviewed literature review examining the rise in loperamide misuse and abuse in the U.S., including trends in reported toxicity cases and associated cardiac risks, published in JAPhA.

54 case reports reviewed (1985–2016) Peer-reviewed

Literature review Toxicology

SCIENCE AND PRACTICE

Journal of the American Pharmacists Association 57 (2017) 545–550

Contents lists available at ScienceDirect

Journal of the American Pharmacists Association

journal homepage: www.japha.org

RESEARCH

Loperamide misuse and abuse

Heather Miller, Ladan Panahi, Daniel Tapia, Andrew Tran, John D. Bowman*

ARTICLE INFO

Article history:
Received 18 September 2016
Accepted 26 December 2016

ABSTRACT

Objective: The epidemic of opioid prescription deaths in recent years resulted in the recent rescheduling of hydrocodone-containing products to restrict access to them. Opioid users have recognized that loperamide can ameliorate withdrawal symptoms and also produce euphoria in very high doses. This article discusses the potential for loperamide misuse and abuse and examines trends in the increasing number of published cases of loperamide toxicity.
Design: PubMed was used to search MEDLINE for case reports of loperamide abuse.
Setting: United States.
Main outcome measures: Numbers of cases of loperamide misuse, characteristics of patients, reported toxicities.
Results: From 1985 to 2016, 54 case reports of loperamide toxicity were published, with 21 cases between 1985 and 2013 and 33 cases between 2014 and 2016. In addition, 179 cases of intentional loperamide misuse were reported to the National Poison Database System between 2008 and 2016, with more than half reported after January 1, 2014.
Conclusion: Loperamide misuse and abuse is increasing in the United States, and pharmacists are encouraged to monitor and restrict their sales.

Introduction

Opioid misuse has become a growing public concern worldwide. According to the Centers for Disease Control and Prevention, more people died from drug overdose in 2014 than in any year on record, with the majority of these deaths involving opioids.[1] Prescription opioid analgesics appear to be one of the driving forces behind this epidemic, as both sales and deaths associated with prescription opioids have quadrupled since 1999.[1] In 2010, the U.S. Food and Drug Administration (FDA) approved tamper-resistant oxycodone, designed to discourage misuse of the medication.[2] In 2014, the Drug Enforcement Agency rescheduled hydrocodone-containing combination products from Schedule III of the Controlled Substance Act to the more tightly regulated Schedule II in an effort to minimize abuse and misuse potential.[3] As prescription opioids become less accessible, the use of nonprescription preparations, such as loperamide and dextromethorphan, may be on the rise.[4-6]

Loperamide is a commonly used antidiarrheal sold in the United States as a generic or under the trade name Imodium®. It is available with or without a prescription in tablet, capsule, and liquid forms at a recommended dose not exceeding 8 mg/day (nonprescription) or 16 mg/day (prescription). Loperamide is an intestinal μ-opioid receptor agonist thought to have little misuse potential[7]; however, published reports of misuse and overdose have increased in recent years. There have been growing discussions on Internet forums about using high doses of loperamide to alleviate symptoms of opioid withdrawal and to achieve feelings of euphoria.[8-11] The 2014 Report of the American Association of Poison Control Centers notes 1230 case mentions for loperamide, including 887 single exposures and 2 deaths.[12] More recently, multiple cases of cardiac dysrhythmias and prolongation of the QRS and QTc intervals have been linked to loperamide misuse.[13] An analysis of the National Poison Database System identified 179 cases of loperamide misuse from January 1, 2008, to March 31, 2016 (Figure 1).[14] In June 2016, the FDA issued a warning about serious heart problems associated with high doses of loperamide. The FDA urges patients to follow package instructions when using loperamide and health care professionals to consider loperamide as a possible cause of cardiac events.[15]

Objective

This article discusses the potential for loperamide misuse and examines trends in the increasing number of published cases of loperamide toxicity.

Disclosure: The authors declare no conflicts of interest or financial interests in any product or service mentioned in this article.

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E-mail address: bowman@pharmacy.tamhsc.edu (J.D. Bowman).

http://dx.doi.org/10.1016/j.japh.2016.12.079

Teaching & speaking

Presentations

Click or tap a thumbnail to view

Warfarin: Mechanism, Dosing & Monitoring

Baylor Scott & White Health — Temple Anticoagulation Clinic · March 2025

Provider-facing education on warfarin's mechanism of action, dosing and INR monitoring, and drug/disease/dietary interactions.

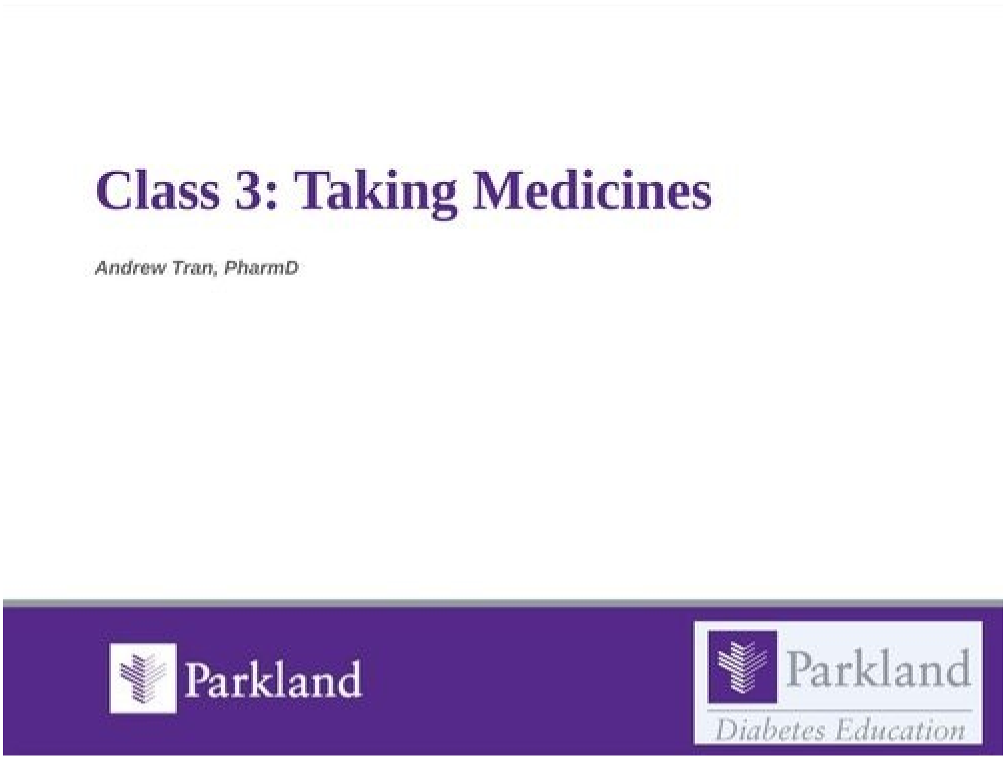
AnticoagulationProvider education

Healthy Living with Diabetes, Class 3: Taking Medicines

Patient education series · 2021

Patient-facing class on diabetes medications — how they work, insulin and injectable basics, storage, avoiding mix-ups, managing missed doses, and using MyChart for refills.

Patient educationDiabetes

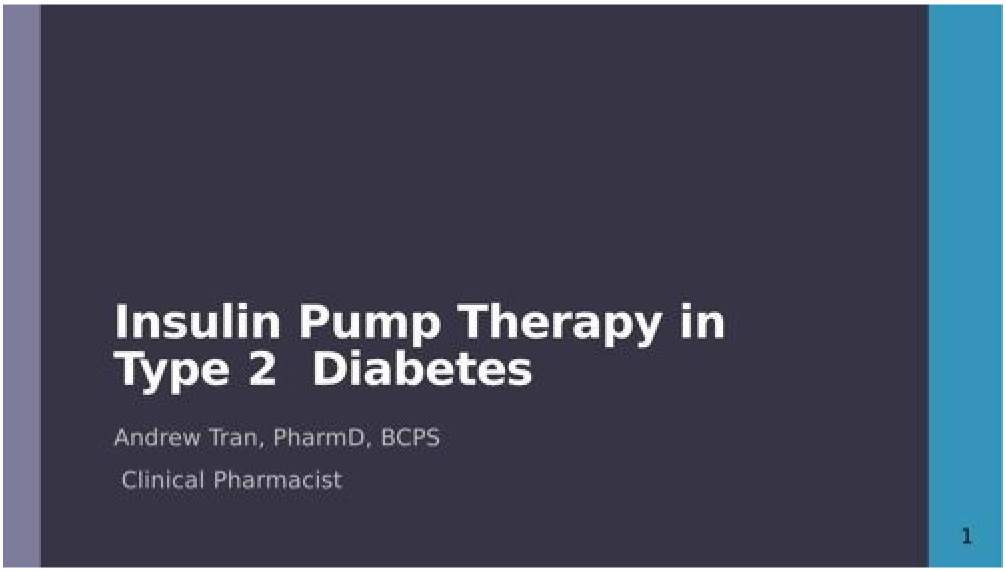


Insulin Pump Therapy in Type 2 Diabetes

Baylor Scott & White Health · 2020

Clinical review of insulin pump therapy in T2DM, including a detailed patient case with basal-rate titration.

DiabetesInsulin therapy

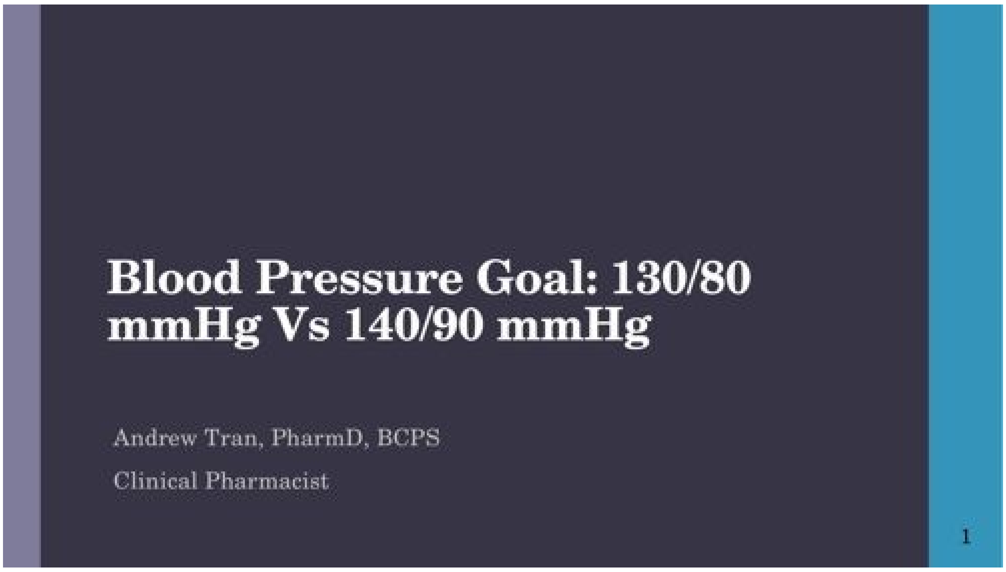


Blood Pressure Goal: 130/80 vs. 140/90 mmHg

Baylor Scott & White Health · 2020

Case-based comparison of competing hypertension guideline targets, closing on individualized, patient-centered goal-setting.

HypertensionGuideline review



Update to Migraine Therapy: New Injectable Preventative Medications

Penobscot Community Health Care · May 23, 2019

Provider CE lecture on updated migraine treatment and prevention guidelines, covering the new anti-CGRP injectable preventatives (erenumab, galcanezumab, fremanezumab), their mechanism of action, and patient selection.

NeurologyProvider education

Pharmacist-led DOAC Telephone Clinic in an FQHC

Maine Society of Health-System Pharmacists · April 6, 2019

Residency research project evaluating direct oral anticoagulant use and the implementation of a pharmacist-led DOAC telephone clinic in an outpatient federally qualified health center.

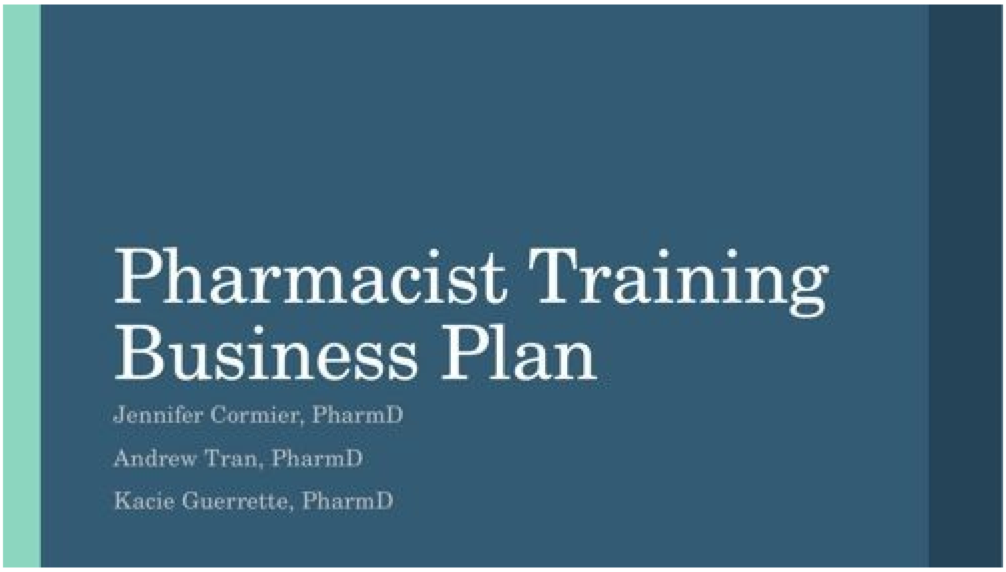
Residency researchAnticoagulation

Pharmacist Training Business Plan

Penobscot Community Health Care · 2019 · co-authored with Jennifer Cormier, PharmD and Kacie Guerrette, PharmD

PGY-1 orientation and competency framework, including a 5-week onboarding schedule and competency checklist.

Residency program design

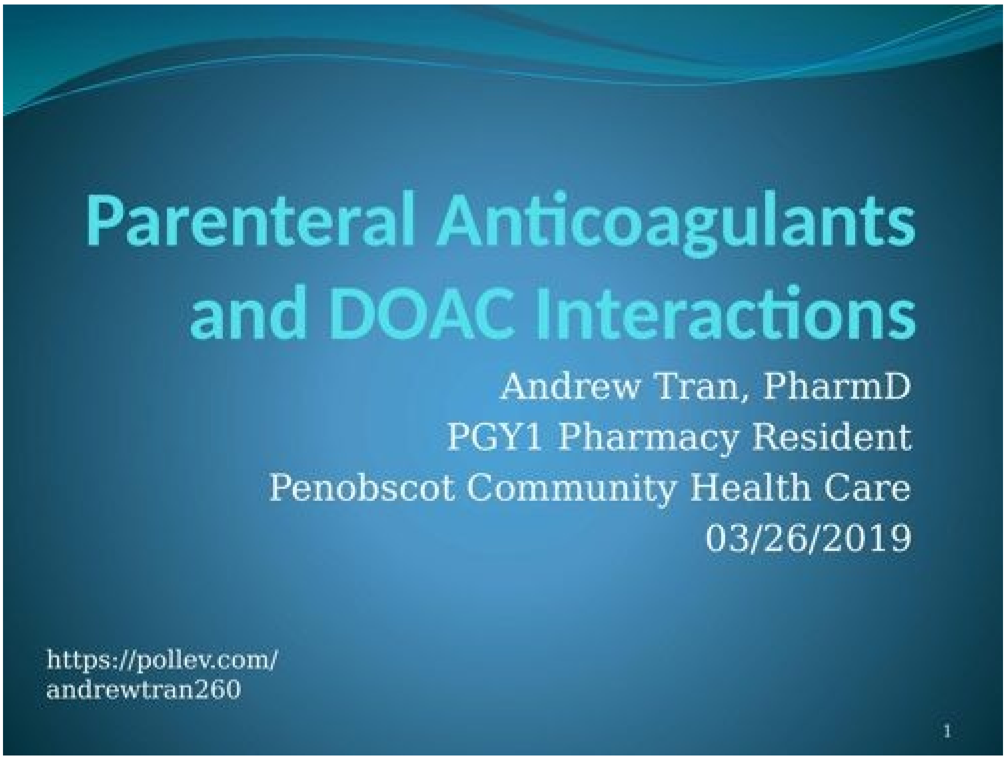


DOAC & Parenteral Anticoagulant Interactions

Penobscot Community Health Care · March 26, 2019

Provider CE lecture on direct oral anticoagulants and parenteral anticoagulants, delivered as PGY-1 pharmacy resident.

AnticoagulationProvider education

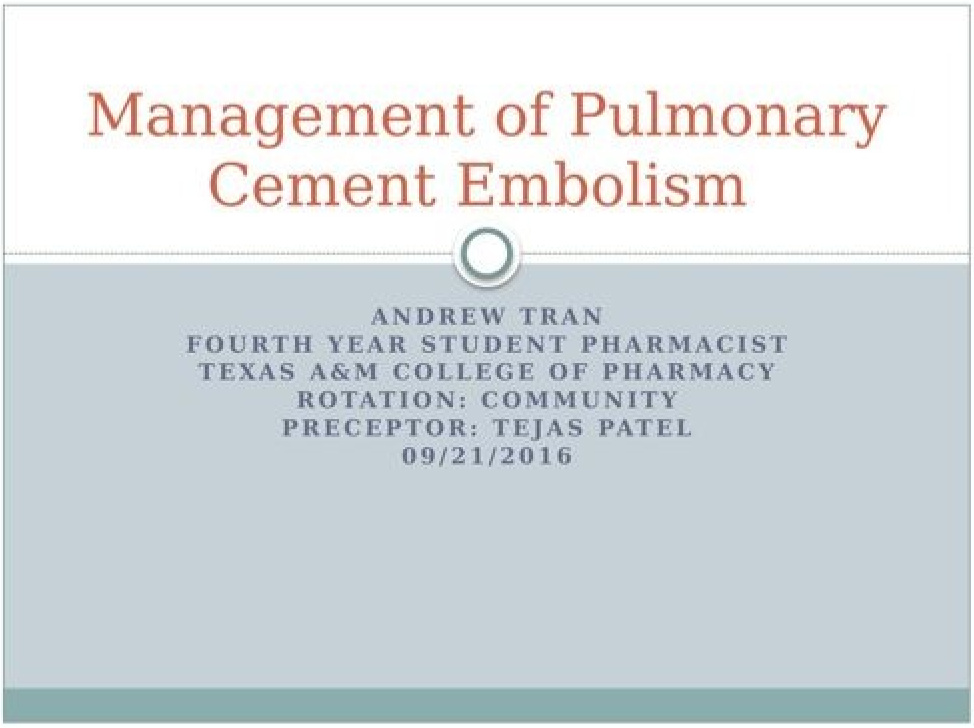


Management of Pulmonary Cement Embolism

Texas A&M College of Pharmacy — Community rotation · September 21, 2016

Case-based review of pulmonary cement embolism following vertebroplasty, presented as a 4th-year student pharmacist.

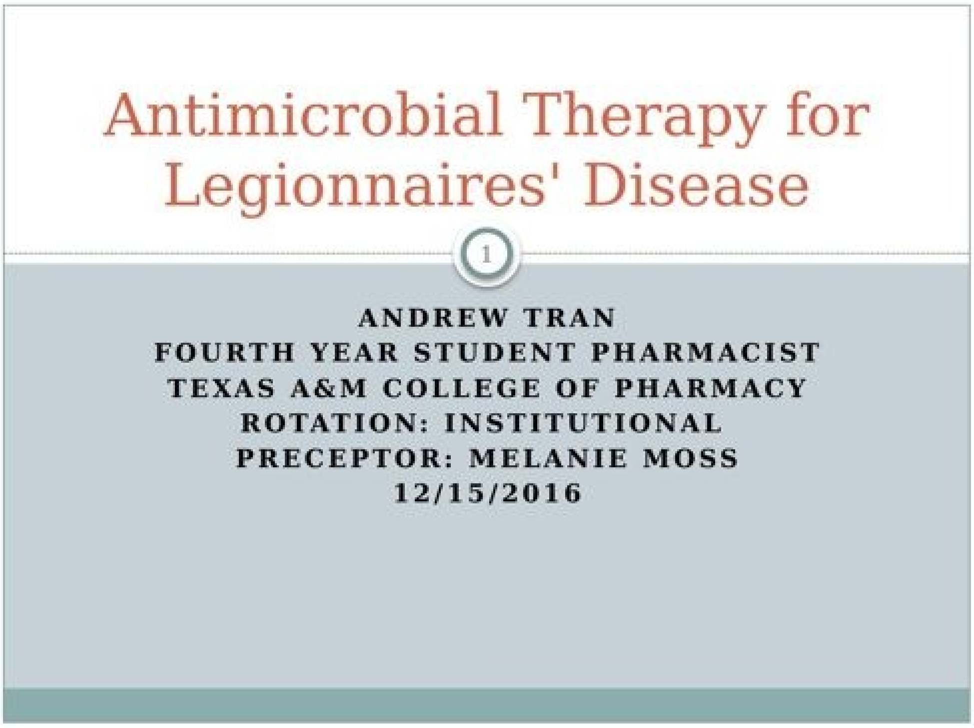
Case presentation



Antimicrobial Therapy for Legionnaires' Disease

Texas A&M College of Pharmacy — Institutional rotation · December 15, 2016

Patient case covering risk factors, diagnostic testing, and antimicrobial regimen selection for Legionnaires' disease, presented as a 4th-year student pharmacist.
Case presentationInfectious disease



Where it started

Residency Training

Before any of the work above, this is where I learned how to practice ambulatory care pharmacy. I completed my PGY-1 Community Pharmacy Residency (Emphasis in Ambulatory Care) at Penobscot Community Health Care (PCHC), a Federally Qualified Health Center in Bangor, Maine, from June 2018 to June 2019 — a mix of core clinical rotations, longitudinal committee work, and a residency research project.

Core & Elective Learning Experiences
Ambulatory Care I
Penobscot Community Health Care (PCHC) · Bangor, ME
Jul – Sep 2018 · Preceptor: Jessica Bates, PharmD, BCACP
Foundational rotation delivering direct, patient-centered chronic disease management within an FQHC primary care setting, with concentrated focus on anticoagulation and diabetes care.
Ambulatory Care II

Helen Hunt Health Center (HHHC) · Old Town, ME
Sep – Oct 2018 · Preceptor: Meagan Rusby, PharmD
Independent management of chronic disease states including diabetes, hypertension, hyperlipidemia, hepatitis C, and smoking cessation, with progressively increasing autonomy over patient visits.

Ambulatory Care III
St. Joseph's Internal Medicine Clinic (SJIM) · Bangor, ME
Oct – Nov 2018 · Preceptor: Eva Quirion, NP, PhD
Embedded ambulatory care rotation within a primary care practice, prioritizing provider referrals across diabetes, hypertension, anticoagulation, asthma, and COPD, with an emphasis on interdisciplinary communication.

Hepatitis C Clinic
Brewer Medical Center (BMC) · Brewer, ME
Dec 2018 – Jan 2019 · Preceptor: Katie Sawicki, PharmD
Dedicated rotation managing patients on direct-acting antiviral therapy for hepatitis C, including treatment selection, monitoring, and coordination of care with prescribers.

Transitions of Care – Home Visits
Penobscot Community Health Care (PCHC) · Bangor, ME
Nov – Dec 2018 · Preceptor: Matthew Christie, PharmD, BCACP
Conducted comprehensive medication reviews, home visits, and hospital follow-up visits for recently discharged patients, working alongside nurse care managers and social workers to safely bridge hospital-to-home transitions.

Pharmacy Administration – 340B & Management
Penobscot Community Health Care (PCHC) · Bangor, ME
Jan – Feb 2019 · Preceptor: Frank McGrady, PharmD, BCPS
Operations rotation with the Director of Pharmacy covering community pharmacy management, 340B program compliance and finances, and Accountable Care Organization structure within an FQHC.

Elective – Specialty Infusion Pharmacy
Penobscot Community Health Care (PCHC) · Bangor, ME
Feb – Apr 2019 · Preceptor: Tiffany Carlisle, PharmD
Managed infusion therapy for patients with autoimmune and inflammatory conditions (Crohn's, rheumatoid arthritis, psoriasis, ulcerative colitis, multiple sclerosis), reviewing orders for dosing, drug interactions, and pre-medication needs.

Elective – Academia

Husson University School of Pharmacy · Bangor, ME

May – Jun 2019 · Preceptor: Frank McGrady, PharmD, BCPS

Introduction to the pharmacist-faculty role, covering ACPE accreditation standards, curriculum development, and experiential education alongside Husson School of Pharmacy faculty.

Longitudinal Experiences

Controlled Substance Stewardship Committee (CSS)

Penobscot Community Health Care (PCHC) · Bangor, ME

Jun 2018 – Jun 2019

Weekly interdisciplinary case review of patients on controlled substances alongside psychiatry, pain management, and social work — developing taper plans and evidence-based recommendations for prescribers.

Pharmacy & Therapeutics Committee (P&T)

Penobscot Community Health Care (PCHC) · Bangor, ME

Jun 2018 – Jun 2019

Participated in formulary review and medication-use policy decisions as a longitudinal member of the P&T committee.

Staffing & Immunization Administration

Brewer Medical Center Pharmacy (BMC) · Brewer, ME

Jun 2018 – Jun 2019

Weekly staffing in a 340B outpatient community pharmacy — prescription verification, immunization administration, and medication therapy management services.

Teaching Certificate Program

University of New England College of Pharmacy (UNE) · Portland, ME

Jun 2018 – Jun 2019

Completed a longitudinal teaching certificate program, precepting and co-precepting APPE and IPPE students throughout the residency year and developing a personal teaching philosophy.

Residency Research Project

Evaluation of Direct Oral Anticoagulant Use and Implementation of a Pharmacist-Led DOAC Telephone Clinic in an Outpatient Federally Qualified Health Center

Penobscot Community Health Care, Bangor, ME · Jun 2018 – Jun 2019 · Co-authored with Jessica Bates, PharmD, BCACP · Presented at ASHP Midyear Clinical Meeting &

MSHP Spring Conference

Designed and evaluated a pharmacist-led telephone clinic for DOAC management, presented as the capstone research project of the residency year. See the full poster under Projects.

View poster ↗

What colleagues say

Testimonials

"Andrew went above and beyond during the systemwide downtime and helped a new patient during enrollment by calling and explaining everything to both the patient and their daughter. Both came into the clinic to fill out paperwork and had nothing but praise for Andrew. Thank you, Andrew, for all that you do for our patients — it doesn't go unnoticed."

DV

Donna Valerio

Operations Manager, Baylor Scott & White Health

"You have taken on duties in addition to your assignments, and you've been very prompt and helpful with requests. Your commitment to the team shows — thank you for all that you do at the clinic to support remote work. I am very grateful to have you as part of the team."

AN

Aimee Nguyen, PharmD, BCGP

Clinical Pharmacist, Baylor Scott & White Health

"Andrew, congratulations on your service award. You have become an important part of our team. We are excited to have you here full time. You have shown that you are a great team player."

KB

Kimberly Baty, PharmD, BCPS

Pharmacist Supervisor, Baylor Scott & White Health

Have a role or project in mind?

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